

Seattle Gender Wellness Center

1420 5th Ave, Suite 3100 · Seattle, WA 98101 · (206) 555-0192

PATIENT

Rivera, Alex
DOB: 08/12/1995 (age 27) · she/her
MRN: GWC-2023-00447

VISIT

Date: January 15, 2023
Provider: Dr. Sarah Chen, MD — Endocrinology
Type: New Patient · Gender-Affirming Care Initiation

& **ALLERGIES: Penicillin (rash/urticaria) · Sulfonamides (hives, facial swelling)**

SUBJECTIVE

Alex is a 27-year-old transgender woman presenting for initiation of gender-affirming hormone therapy (GAHT). She has been living full-time for 18 months and has been working with a gender-affirming therapist for 2 years (Dr. Patel, Emerald City Counseling). She is well-informed regarding the expected timeline and risks of feminizing HRT and meets DSM-5 criteria for gender dysphoria. She has not used exogenous hormones previously. She denies personal or family history of thromboembolic disease, cardiovascular disease, or hormone-sensitive malignancy.

Review of Systems: Negative for chest pain, palpitations, dyspnea, leg swelling, severe headaches, or visual changes.

OBJECTIVE — VITAL SIGNS

BP: 122/78 mmHg HR: 72 bpm Weight: 165 lbs Height: 5' 8" BMI: 25.1

OBJECTIVE — BASELINE LABORATORY RESULTS

TEST	RESULT	UNIT	FLAG
Estradiol (E2)	45	pg/mL	— baseline
Testosterone, Total	480	ng/dL	— pre-HRT
Prolactin	7.2	ng/mL	normal
LH	5.8	mIU/mL	normal
FSH	4.2	mIU/mL	normal
Hemoglobin	14.8	g/dL	normal
Potassium	4.1	mEq/L	normal
Creatinine	0.9	mg/dL	normal
ALT	28	U/L	normal
AST	24	U/L	normal

ASSESSMENT & PLAN

1. Gender-Affirming Hormone Therapy — Initiated

Starting Estradiol valerate 2 mg sublingual twice daily.
Starting Spironolactone 100 mg daily (anti-androgen, testosterone suppression).
Expected timeline: breast development begins 3–6 months; full feminization 2–3 years.
Counseled on fertility: referred to Pacific NW Reproductive Medicine for sperm banking if desired.
Discussed risks: DVT/PE (small increased risk), weight redistribution, emotional changes.

2. Monitoring Plan

Labs in 3 months (April 2023): estradiol, testosterone, potassium, ALT/AST, CBC.
HRT targets: Estradiol 100–300 pg/mL. Testosterone <50 ng/dL.
Monitor potassium closely — spironolactone can cause hyperkalemia.
Avoid NSAIDs and high-potassium foods in excess while on spironolactone.

3. Safety Counseling

Reviewed DVT warning signs. Patient instructed to present to ED for sudden leg pain, shortness of breath, or chest pain.
Advised against smoking or combined oral contraceptive use while on estradiol.
Emergency contact: Trans Lifeline 877-565-8860.

Follow-up: 3 months (April 2023)

Electronically signed by: Dr. Sarah Chen, MD
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